

SUPERVISED VOCATIONAL QUALIFICATION DIPLOMA - EXAMINATION

Session: Revision Mock Set 03	Duration: 3 hours
Specialty: Psychotherapy	Coefficient: 04
Eliminatory mark: < 8/20	Training reference: VTI-AEDA examination style

COMPREHENSIVE VOCATIONAL PAPER - SOLUTION / MARKING GUIDE PRACTICE SET 03

Revision material based on the supplied VTI-AEDA 2025 examination format and the supplied psychotherapy course recordings. Not an official examination paper.

How to use this marking guide

For revision, first attempt the paper without notes. Then compare not only the answer letter but the clinical cue that makes the answer correct. For legal/ethical items, the guide follows the concepts emphasized by the supplied course/examination materials; in real practice, applicable Cameroon law, institutional policy and the current professional code must also be checked.

PART ONE: PSYCHOPATHOLOGY - ANSWERS

1. A therapist notes that Kemi has perfectionism, orderliness, rigidity and control that interfere with flexibility and task completion. Which condition should be considered first from the options below?

- A. Substance use disorder
- B. Schizoid personality disorder
- C. Obsessive-compulsive personality disorder (OCPD)**
- D. Major depressive disorder

Answer: C - Obsessive-compulsive personality disorder (OCPD). OCPD is a personality pattern of perfectionism and control; unlike OCD, true obsessions and compulsions are not required.

Memory aid: OCPD = Order, Control, Perfectionism, Duty.

2. A therapist notes that Lucas has detachment from social relationships and restricted emotional expression with little desire for close relationships. Which condition should be considered first from the options below?

- A. Generalized anxiety disorder (GAD)
- B. Substance use disorder
- C. Schizophrenia
- D. Schizoid personality disorder**

Answer: D - Schizoid personality disorder. The key pattern is social detachment and limited affect, rather than fear of rejection as in avoidant personality.

Memory aid: SCHIZOID = Solitary by preference.

3. Hassan, a 34-year-old sales representative, presents with excessive need to be cared for, difficulty making decisions independently, submissiveness and fear of separation. Which diagnosis or clinical concept best fits this presentation?

- A. Bipolar disorder
- B. Agoraphobia
- C. Dependent personality disorder**
- D. Opioid intoxication

Answer: C - Dependent personality disorder. Dependent personality is characterized by reliance on others for decisions, reassurance and support.

Memory aid: DEPENDENT = Decisions Entrusted to People; separation fear.

4. Owen, a 27-year-old nurse, presents with persistent social-communication difficulties together with restricted or repetitive patterns of behavior or interests beginning in development. Which diagnosis or clinical concept best fits this presentation?

- A. Anorexia nervosa, binge-eating/purging type

B. Panic disorder

C. Autism spectrum disorder

D. Schizophrenia

Answer: C - Autism spectrum disorder. Autism requires both social-communication differences and restricted/repetitive behavior patterns.

Memory aid: 2R domains: Relating/communication + Repetitive/restricted behavior.

5. Carlos, a 37-year-old driver, presents with recurrent unexpected panic attacks followed by persistent worry about more attacks or maladaptive avoidance. Which diagnosis or clinical concept best fits this presentation?

A. Schizophrenia

B. Post-traumatic stress disorder (PTSD)

C. Generalized anxiety disorder (GAD)

D. Panic disorder

Answer: D - Panic disorder. Panic disorder is more than a single panic attack: recurrent unexpected attacks are followed by ongoing concern or behavior change.

Memory aid: PANIC: Panic Attacks + New worry/avoidance Is Continuing.

6. During assessment, Xavier reports excessive, difficult-to-control worry across several life areas for about six months, with tension, restlessness or poor sleep. Which of the following is the BEST match?

A. Schizoid personality disorder

B. Substance use disorder

C. Binge-eating disorder

D. Generalized anxiety disorder (GAD)

Answer: D - Generalized anxiety disorder (GAD). GAD is characterized by persistent generalized worry across multiple domains rather than fear tied to one object or situation.

Memory aid: GAD = General Anxiety across Domains.

7. Esther, a 53-year-old civil servant, presents with marked fear of a specific object or situation that triggers immediate anxiety and avoidance out of proportion to the actual danger. Which diagnosis or clinical concept best fits this presentation?

A. Anorexia nervosa, binge-eating/purging type

B. Substance use disorder

C. Bulimia nervosa

D. Specific phobia

Answer: D - Specific phobia. A specific phobia is circumscribed: the fear is linked to a particular object or situation and typically produces avoidance.

Memory aid: SPECIFIC = one SPECIFIC trigger.

8. During assessment, Irene reports marked fear of social or performance situations because of possible scrutiny, embarrassment or negative evaluation. Which of the following is the BEST match?

A. Schizophrenia

B. Major depressive disorder

C. Bipolar disorder

D. Social anxiety disorder

Answer: D - Social anxiety disorder. Social anxiety centers on being observed, judged or embarrassed in social/performance settings.

Memory aid: SOCIAL = Scrutiny Of Client In Activities with Lots of people.

9. A therapist notes that Owen has fear and avoidance of situations where escape may be difficult or help unavailable, such as crowds, public transport or being outside alone. Which condition should be considered first from the options below?

A. Binge-eating disorder

B. Anorexia nervosa, binge-eating/purging type

C. Agoraphobia

D. Substance use disorder

Answer: C - Agoraphobia. Agoraphobia concerns situations in which escape or access to help feels difficult; it may occur with or without panic disorder.

Memory aid: AGORA = *A*fraid to *G*o *O*ut/*R*isk being *A*way from *h*elp.

10. During assessment, Esther reports intrusive unwanted thoughts or urges and/or repetitive behaviors or mental acts performed to reduce anxiety. Which of the following is the BEST match?

A. Post-traumatic stress disorder (PTSD)

B. Obsessive-compulsive disorder (OCD)

C. Obsessive-compulsive personality disorder (OCPD)

D. Specific phobia

Answer: B - Obsessive-compulsive disorder (OCD). OCD involves obsessions (intrusive thoughts/urges/images) and compulsions (repetitive acts or mental rituals).

Memory aid: O = *Obsession is a thought*; C = *Compulsion is a Conduct*.

SECTION B: TRUE/FALSE - EXPLANATIONS

1. FALSE. Schizoid personality disorder is defined primarily by intense fear of criticism and rejection.

Schizoid personality is primarily social detachment with limited desire for close relationships.

2. TRUE. Autism spectrum disorder includes both social-communication difficulties and restricted/repetitive behavior patterns.

Both domains are required in the modern diagnostic framework.

3. TRUE. The four D framework can include deviance, distress, dysfunction and danger.

These four dimensions are commonly taught as aids for thinking about abnormality.

4. TRUE. Comorbidity means that two or more disorders can occur in the same person.

Comorbidity is the co-occurrence of disorders.

5. FALSE. A specific phobia is generalized worry across many areas of life.

Generalized worry across many domains is more characteristic of GAD.

SECTION C: MODEL SHORT ANSWERS

1. Differentiate OCD from obsessive-compulsive personality disorder. **(2mks)**

- OCD involves obsessions and/or compulsions causing distress/time burden.

- OCPD is a pervasive pattern of perfectionism, orderliness and control; obsessions/compulsions are not required.

Memory aid: OCD = *obsessions/compulsions*; OCPD = *order/control/personality*.

2. Give two signs that suggest psychosis. **(2mks)**

- Examples: delusions, hallucinations, markedly disorganized speech/behavior, impaired reality testing.

Memory aid: 3D: *Delusions, perceptual Disturbance, Disorganization*.

3. Why must suicide risk be assessed in a severely depressed client? **(2mks)**

- Depression may involve hopelessness and suicidal ideation; assessment identifies immediacy of risk and need for protective intervention/referral.

Memory aid: Ask directly about thoughts, plan, intent, means and protective factors.

4. State two differences between GAD and a specific phobia. **(2mks)**

- GAD: diffuse worry across multiple domains; specific phobia: circumscribed fear of a particular object/situation.

- GAD is persistent generalized worry; phobia is triggered by the feared stimulus and avoidance.

Memory aid: General vs specific.

5. What does DSM refer to in psychopathology? **(2mks)**

- Diagnostic and Statistical Manual of Mental Disorders.

- A classification system used to describe diagnostic criteria; it supports but does not replace clinical assessment.

Memory aid: DSM = *Diagnostic + Statistical + Manual*.

PART TWO: THEORIES/TECHNIQUES - ANSWERS

1. In a session with Carlos, the counselor emphasizes the therapist explains or offers meaning about patterns, defenses, dreams, resistance or behavior. This BEST illustrates:

- A. Flooding
- B. Systematic desensitization
- C. Operant conditioning

D. Interpretation

Answer: D - Interpretation. Interpretation is the analyst's attempt to make latent or previously unconscious meaning more understandable to the client.

Memory aid: *Interpretation = therapist offers possible meaning.*

2. In a session with Wendy, the counselor emphasizes the client unconsciously avoids material or processes that threaten to bring unacceptable conflict into awareness. This BEST illustrates:

- A. Rational Emotive Behavior Therapy (REBT)
- B. Free association
- C. Exposure therapy

D. Resistance

Answer: D - Resistance. Resistance is the defensive blocking of threatening material or therapeutic work.

Memory aid: *Resistance = psychological brakes.*

3. A therapist working with Uma uses an intervention characterized by the client redirects feelings, expectations or relationship patterns from important past relationships onto the therapist. Which approach or technique is being used?

- A. Systematic desensitization
- B. Negative reinforcement
- C. Cognitive behavioral therapy (CBT)

D. Transference

Answer: D - Transference. Transference originates primarily from the client's earlier relationships and becomes enacted in the therapeutic relationship.

Memory aid: *TRANSference = client TRANSfers old relationship feelings to therapist.*

4. A therapist working with Brenda uses an intervention characterized by the therapist's emotional reactions to a client, especially reactions influenced by the therapist's own history or unresolved issues. Which approach or technique is being used?

- A. Gestalt therapy
- B. Countertransference**
- C. Person-centered therapy
- D. Negative reinforcement

Answer: B - Countertransference. Countertransference concerns the therapist's responses to the client and must be monitored through self-awareness and supervision.

Memory aid: *COUNTER = counselor side of the relational reaction.*

5. A therapist working with Victor uses an intervention characterized by therapy emphasizing unconscious conflict, early relationships, defenses, transference, resistance and insight. Which approach or technique is being used?

- A. Psychoanalytic therapy**
- B. Rational Emotive Behavior Therapy (REBT)
- C. Systematic desensitization
- D. Classical conditioning

Answer: A - Psychoanalytic therapy. Psychoanalytic approaches seek insight into unconscious patterns and how early relationships repeat in present life and therapy.

Memory aid: *FIRT: Free association, Interpretation, Resistance, Transference.*

6. In a session with Kemi, the counselor emphasizes a goal-directed, socially embedded approach emphasizing conscious choice, responsibility, lifestyle and movement from inferiority toward useful belonging. This BEST illustrates:

- A. Adlerian therapy**
- B. Systematic desensitization
- C. Progressive muscle relaxation
- D. Gestalt therapy

Answer: A - Adlerian therapy. Adlerian therapy views behavior as purposeful and socially related, with emphasis on goals, choice and responsibility. This is explicitly reflected in the opening lecture of the fused audio.

Memory aid: GSCR: Goal-directed, Social relatedness, Conscious choice, Responsibility.

7. In a session with Patricia, the counselor emphasizes a non-directive relationship based on accurate empathy, unconditional positive regard and congruence. This BEST illustrates:

- A. Interpretation
- B. Dialectical Behavior Therapy (DBT)
- C. Person-centered therapy**
- D. Retroflexion

Answer: C - Person-centered therapy. Rogers' person-centered therapy relies on core facilitative conditions rather than therapist-imposed solutions.

Memory aid: EUC: Empathy, Unconditional positive regard, Congruence.

8. In a session with Uma, the counselor emphasizes a structured approach linking thoughts, emotions and behaviors and using cognitive and behavioral change strategies. This BEST illustrates:

- A. Cognitive behavioral therapy (CBT)**
- B. Social skills training
- C. Free association
- D. Operant conditioning

Answer: A - Cognitive behavioral therapy (CBT). CBT targets maladaptive thinking and behavior through collaborative, structured methods such as cognitive restructuring and behavioral experiments.

Memory aid: TEA: Thoughts - Emotions - Actions.

9. Which psychotherapy concept is most accurately described by the following: identifying inaccurate or dysfunctional thoughts, examining evidence and developing more balanced alternatives?

- A. Dialectical Behavior Therapy (DBT)
- B. Extinction
- C. Cognitive restructuring**
- D. Retroflexion

Answer: C - Cognitive restructuring. Cognitive restructuring is a core CBT method for testing and modifying distorted or unhelpful cognitions.

Memory aid: Catch it, Check it, Change it.

10. In a session with Victor, the counselor emphasizes Albert Ellis' approach that identifies irrational beliefs between events and emotional consequences and actively disputes those beliefs. This BEST illustrates:

- A. Rational Emotive Behavior Therapy (REBT)**
- B. Existential therapy
- C. Cognitive restructuring
- D. Systematic desensitization

Answer: A - Rational Emotive Behavior Therapy (REBT). REBT uses the ABC model and disputation to replace rigid irrational beliefs with more rational alternatives.

Memory aid: ABCDE: Activating event, Belief, Consequence, Dispute, Effective new belief.

11. A therapist working with Victor uses an intervention characterized by an REBT imagery exercise in which the client vividly imagines a difficult event, experiences the emotion, then practices changing unhealthy emotion by changing beliefs. Which approach or technique is being used?

A. Rational emotive imagery

B. Person-centered therapy

C. Psychoanalytic therapy

D. Exposure therapy

Answer: A - Rational emotive imagery. Rational emotive imagery is an experiential REBT technique for rehearsing healthier emotional responses through belief change.

Memory aid: *Imagine - feel - dispute - re-feel.*

12. In a session with Xavier, the counselor emphasizes a here-and-now experiential approach emphasizing awareness, contact, unfinished business and integration. This BEST illustrates:

A. Gestalt dream work

B. Adlerian therapy

C. Gestalt therapy

D. Cognitive restructuring

Answer: C - Gestalt therapy. Gestalt therapy focuses on present awareness and experiments that help clients own and integrate experience.

Memory aid: *Gestalt = awareness in the NOW.*

13. Which psychotherapy concept is most accurately described by the following: the client speaks to an imagined person or part of self placed in an empty chair, often switching roles to deepen awareness?

A. Empty-chair technique

B. Flooding

C. Free association

D. In vivo exposure

Answer: A - Empty-chair technique. The empty-chair experiment is associated with Gestalt work on unfinished business, polarities and present-moment experience.

Memory aid: *Empty chair = make the absent relationship present.*

14. Which psychotherapy concept is most accurately described by the following: the client re-enacts a dream in the present and may speak as different people, objects or elements of the dream?

A. Classical conditioning

B. Adlerian therapy

C. Rational Emotive Behavior Therapy (REBT)

D. Gestalt dream work

Answer: D - Gestalt dream work. Rather than imposing a fixed interpretation, Gestalt dream work invites the client to own and experience parts of the dream.

Memory aid: *Dream it, become it, speak it.*

15. A therapist working with Brenda uses an intervention characterized by turning back onto oneself what one wants to do to another, or doing to oneself what one wishes another would do. Which approach or technique is being used?

A. Cognitive behavioral therapy (CBT)

B. Classical conditioning

C. Retroflexion

D. Gestalt therapy

Answer: C - Retroflexion. Retroflexion is a Gestalt contact-boundary disturbance directed back toward the self.

Memory aid: *RETRO = return action back to self.*

SECTION B: TRUE/FALSE - EXPLANATIONS

1. TRUE. The empty-chair technique is commonly associated with Gestalt therapy.

It is a classic Gestalt experiment for unfinished business or polarities.

2. FALSE. Existential therapy aims to eliminate every form of anxiety from life.

Existential therapy accepts that some anxiety is inherent in freedom, responsibility and existence; the aim is authentic engagement, not zero anxiety.

3. TRUE. Adlerian therapy emphasizes purposeful, goal-directed behavior and personal responsibility.

Those ideas are explicitly emphasized in the course audio opening.

4. TRUE. CBT assumes thoughts, emotions and behaviors can influence one another.

This reciprocal link is a core CBT formulation.

5. TRUE. DBT teaches mindfulness, distress tolerance, emotion regulation and interpersonal effectiveness.

These are the four major DBT skills domains.

SECTION C: MODEL SHORT ANSWERS

1. What is transference? (2mks)

- Client projects/re-enacts feelings, expectations or patterns from earlier important relationships onto the therapist.

Memory aid: *Client TRANSfers old patterns.*

2. What is countertransference? (2mks)

- Therapist emotional response to the client, especially where the therapist's own history or vulnerabilities shape that response; requires awareness/supervision.

Memory aid: *Counter = counselor side.*

3. State one goal of existential therapy. (1mks)

- Help the client face freedom/responsibility, clarify meaning and values, make authentic choices and live more deliberately - any one.

Memory aid: *FMRC.*

PART THREE: ETHICAL/LEGAL - ANSWERS

1. A counselor asks which principle or professional concept is defined by keeping commitments, maintaining trust and being faithful to professional responsibilities. The BEST answer is:

- A. Mandated reporting
- B. Malpractice
- C. Fidelity**
- D. Subpoena response

Answer: C - Fidelity. Fidelity concerns loyalty, promises and trust within professional relationships.

Memory aid: *Fidelity = faithful to promises.*

2. During supervision, Daniel describes an ongoing process in which the client receives understandable information about services, risks, benefits, alternatives, limits of confidentiality and choice. Which concept should guide the discussion?

- A. Fidelity
- B. Mandated reporting
- C. Professional competence
- D. Informed consent**

Answer: D - Informed consent. Informed consent is not merely a signature; it is an ongoing communication process supporting voluntary, informed participation.

Memory aid: *BRALC: Benefits, Risks, Alternatives, Limits, Choice.*

3. A counselor asks which principle or professional concept is defined by the professional duty to protect information learned in the therapeutic relationship, subject to recognized legal and ethical exceptions. The BEST answer is:

- A. Termination and referral
- B. Mandated reporting
- C. Confidentiality**
- D. Gift decision-making

Answer: C - Confidentiality. Confidentiality protects client information but is not absolute; limits should be explained early.

Memory aid: *Confidentiality = keep private, know the exceptions.*

4. In professional practice, a situation calls for the client's broader right to control access to self, body and personal information. Which ethical/legal concept is MOST directly involved?

- A. Dual or multiple relationship
- B. Ethical response to colleague misconduct
- C. Privacy**
- D. Justice

Answer: C - Privacy. Privacy is the client's right to be left alone or control access, whereas confidentiality is the professional's duty regarding information received.

Memory aid: *Privacy belongs to the person; confidentiality binds the professional.*

5. A counselor asks which principle or professional concept is defined by a legal protection that may prevent compelled disclosure of certain therapy communications in judicial proceedings, depending on jurisdiction. The BEST answer is:

- A. Subpoena response
- B. Privileged communication**
- C. Beneficence
- D. Veracity

Answer: B - Privileged communication. Privilege is a legal evidentiary concept and is distinct from the broader ethical duty of confidentiality.

Memory aid: *Privilege = courtroom/legal shield.*

6. In professional practice, a situation calls for taking reasonable protective action when a client presents a serious, credible threat toward an identifiable person, as required by applicable law and ethics. Which ethical/legal concept is MOST directly involved?

- A. Record keeping
- B. Duty to warn/protect**
- C. Boundary crossing
- D. Fidelity

Answer: B - Duty to warn/protect. The course tests duty to warn/protect as an exception to confidentiality when serious danger to others is present. Actual legal requirements vary by jurisdiction.

Memory aid: *TIP: Threat serious, Identifiable target, Protect reasonably.*

7. In professional practice, a situation calls for reporting suspected abuse or other legally reportable situations to designated authorities when law or professional duty requires it. Which ethical/legal concept is MOST directly involved?

- A. Mandated reporting**
- B. Sexual relationship with a current client
- C. Privacy
- D. Professional competence

Answer: A - Mandated reporting. Mandated reporting is a recognized limit to confidentiality; therapists should explain this limit during informed consent.

Memory aid: *Report when law makes reporting mandatory.*

8. During supervision, Owen describes professional negligence in which a duty is breached and the breach causes harm. Which concept should guide the discussion?

- A. Malpractice**
- B. Justice
- C. Gift decision-making
- D. Confidentiality

Answer: A - Malpractice. Malpractice requires more than an unfavorable outcome: there must be a professional duty, breach, harm and causal link.

Memory aid: *DBHC: Duty, Breach, Harm, Causation.*

9. A counselor asks which principle or professional concept is defined by practicing only within areas supported by appropriate education, training, supervised experience and current skill. The BEST answer is:

- A. Professional competence**
- B. Confidentiality
- C. Veracity
- D. Gift decision-making

Answer: A - Professional competence. Using techniques without adequate training can expose clients to harm and the professional to ethical or legal liability.

Memory aid: *Know your scope; seek supervision or refer.*

10. In professional practice, a situation calls for a situation in which the therapist has another significant role or relationship with the client in addition to therapy. Which ethical/legal concept is MOST directly involved?

- A. Gift decision-making
- B. Dual or multiple relationship**
- C. Duty to warn/protect
- D. Professional competence

Answer: B - Dual or multiple relationship. Multiple relationships are not automatically harmful in every context, but should be avoided or carefully managed when they risk exploitation, impaired objectivity or harm.

Memory aid: *Two roles = review power, objectivity, exploitation and harm.*

11. In professional practice, a situation calls for a departure from customary therapeutic boundaries that may be clinically appropriate and non-exploitative in context. Which ethical/legal concept is MOST directly involved?

- A. Veracity
- B. Ethical response to colleague misconduct
- C. Boundary crossing**
- D. Professional competence

Answer: C - Boundary crossing. A boundary crossing is not automatically unethical; context, culture, intent and effect matter.

Memory aid: Crossing may be thoughtful; violation causes or risks serious harm/exploitation.

12. During supervision, Wendy describes a serious harmful or exploitative breach of therapeutic boundaries. Which concept should guide the discussion?

- A. Termination and referral
- B. Beneficence
- C. Dual or multiple relationship
- D. Boundary violation**

Answer: D - Boundary violation. Boundary violations exploit the therapeutic relationship or cause significant harm and are unethical.

Memory aid: Violation = harm/exploitation.

13. A counselor asks which principle or professional concept is defined by sexual or romantic involvement with a current therapy client. The BEST answer is:

- A. Sexual relationship with a current client**
- B. Boundary violation
- C. Autonomy
- D. Subpoena response

Answer: A - Sexual relationship with a current client. Sexual relationships with current clients are prohibited because of power imbalance and exploitation risk. Terminating therapy to start a relationship does not erase those concerns.

Memory aid: Current client + sex = no.

14. A counselor asks which principle or professional concept is defined by considering clinical meaning, value, timing, culture, power and exploitation risk before accepting or declining a client gift. The BEST answer is:

- A. Termination and referral
- B. Boundary crossing
- C. Gift decision-making**
- D. Nonmaleficence

Answer: C - Gift decision-making. Gifts require contextual judgment. A culturally meaningful modest gift may be handled differently from an expensive or coercive gift.

Memory aid: VALUE: Value, Understanding/meaning, Local culture, Unequal power, Effect on therapy.

15. A counselor asks which principle or professional concept is defined by accepting goods or services instead of money when ethically permissible, not exploitative and not clinically contraindicated. The BEST answer is:

- A. Subpoena response
- B. Bartering**
- C. Beneficence
- D. Ethical response to colleague misconduct

Answer: B - Bartering. Bartering may be acceptable under some codes when not exploitative and when arrangements are clear and clinically appropriate.

Memory aid: Barter only if Boundaries and Benefits are safe.

SECTION B: MODEL SHORT ANSWERS

1. What is the principle of justice? (2mks)

- Fair and equitable treatment and distribution of benefits, burdens and resources.

Memory aid: *Justice = fairness.*

2. What does duty to warn/protect mean in therapy? (1mks)

- When a serious credible threat to an identifiable person is present, the therapist may have to take reasonable protective action according to applicable law and ethics, even though confidentiality is normally protected.

Memory aid: *TIP: Threat, Identifiable person, Protect.*

3. Define mandated reporting. (1mks)

- A legal/professional requirement to report specified situations such as suspected abuse to designated authorities; it is a limit to confidentiality.

Memory aid: *Mandatory = must report when the law says so.*

4. State two ways a therapist should manage sexual or romantic feelings toward a client. (1mks)

- Acknowledge and monitor the feelings rather than act on them.
- Seek supervision/consultation and strengthen boundaries; refer if objectivity/competence cannot be maintained.

Memory aid: *FEEL, do not ACT: notice, supervise, protect boundaries.*

SECTION C: MODEL SHORT ESSAYS

1. What should a therapist do when credible serious unethical behavior by a colleague is suspected? (2mks)

- Clarify facts, consult the code/supervisor as appropriate, protect the client and follow formal reporting procedures when required.

Memory aid: *CARE: Consult, Assess, Report, Ensure safety.*

2. Differentiate privacy from confidentiality. (2mks)

- Privacy is the client's right to control access to self and personal information.
- Confidentiality is the professional duty to protect information obtained in the professional relationship.

Memory aid: *Privacy = person's right; confidentiality = professional's duty.*

3. Differentiate confidentiality from privileged communication. (1mks)

- Confidentiality is an ethical/professional duty.
- Privilege is a legal evidentiary protection against compelled disclosure in certain proceedings and depends on jurisdiction.

Memory aid: *Privilege = courtroom shield.*

PART FOUR: CULTURE / GROUP / COUNSELING PROCESS / TREATMENT PLAN - ANSWERS

1. In counseling practice, Patricia encounters a situation involving judging other cultures primarily by the standards of one's own culture and treating one's own as the norm. Which concept BEST applies?

- A. Norming stage
- B. Active listening
- C. Progress review

D. Ethnocentrism

Answer: D - Ethnocentrism. Ethnocentrism places one's own cultural frame at the center and may distort assessment or communication.

Memory aid: *ETHNO-centred = my culture at the centre.*

2. In counseling practice, Martha encounters a situation involving pressuring a client to adopt the therapist's or dominant culture's beliefs, values or practices. Which concept BEST applies?

- A. Paraphrasing
- B. Cultural imposition**
- C. Rapport building
- D. Skills-development group

Answer: B - Cultural imposition. Cultural imposition occurs when the helper pushes their own values or practices onto the client.

Memory aid: *IMPOSITION = impose my culture.*

3. A trainee observes using a qualified interpreter rather than relying on a child or untrained friend when language barriers affect care. This is an example of:

- A. Professional interpreter use**
- B. Cultural imposition
- C. Support group
- D. Paraphrasing

Answer: A - Professional interpreter use. Qualified interpreters improve accuracy, confidentiality and informed consent.

Memory aid: *Interpreter = accuracy + confidentiality.*

4. A trainee observes a group emphasizing mutual support, shared experience, acceptance and coping among people facing similar problems. This is an example of:

- A. Self-disclosure
- B. Case formulation
- C. Support group**
- D. Reflection of feeling

Answer: C - Support group. Support groups reduce isolation and offer mutual aid through shared experience.

Memory aid: *SUPPORT = Similar people Understanding and Providing Peer help.*

5. In counseling practice, Nadine encounters a situation involving a structured group focused on teaching information, understanding a condition and coping knowledge. Which concept BEST applies?

- A. Ethnocentrism
- B. Goal setting
- C. Therapeutic confrontation

D. Psychoeducational group

Answer: D - Psychoeducational group. Psychoeducational groups primarily teach information and understanding; they may include discussion and practice.

Memory aid: *EDU = education is central.*

6. Which counseling/group/cultural concept is BEST described by: a group focused on learning and practicing specific behavioral or interpersonal skills?

- A. Group confidentiality
- B. Treatment plan
- C. Skills-development group**
- D. Cognitive-behavioral group

Answer: C - Skills-development group. Skills groups emphasize rehearsal, practice and feedback rather than information alone.

Memory aid: *SKILLS = Show, Keep practicing, Improve, Learn, feedback.*

7. Which counseling/group/cultural concept is BEST described by: a structured group using cognitive and behavioral methods such as thought monitoring, skills practice and behavioral exercises?

- A. Treatment plan
- B. Progress review
- C. Psychoeducational group
- D. Cognitive-behavioral group**

Answer: D - Cognitive-behavioral group. CBT groups combine structured learning with practice and feedback around thoughts and behaviors.

Memory aid: *Group CBT = Think + Do together.*

8. A trainee observes a recurrent functional position adopted by a member, such as scapegoat, monopolizer, peacemaker or spokesperson. This is an example of:

- A. Skills-development group
- B. Therapeutic confrontation
- C. Group role**
- D. Progress review

Answer: C - Group role. Group roles describe patterned ways members participate and influence group dynamics.

Memory aid: *Role = repeated position in the group.*

9. A trainee observes an agreement that members protect information disclosed by other members, while the leader explains that peer confidentiality cannot be guaranteed in the same way as professional confidentiality. This is an example of:

- A. Cultural imposition
- B. Professional interpreter use
- C. Group confidentiality**
- D. Storming stage

Answer: C - Group confidentiality. Confidentiality is essential in groups, but leaders should explain realistic limits because members are not all licensed professionals.

Memory aid: *What is said in group should stay in group - with limits explained.*

10. Which counseling/group/cultural concept is BEST described by: early group stage characterized by orientation, uncertainty, dependence and testing?

- A. Skills-development group
- B. Case formulation
- C. Termination and follow-up
- D. Forming stage**

Answer: D - Forming stage. Forming is the beginning stage in which members learn the purpose, structure and safety of the group.

Memory aid: *FSNPA: Forming, Storming, Norming, Performing, Adjourning.*

11. A trainee observes group stage in which conflict, resistance, competition or emotional testing becomes more visible. This is an example of:

- A. Storming stage**
- B. Treatment plan
- C. Norming stage

D. Group role

Answer: A - Storming stage. Storming often brings disagreement and challenges to roles or leadership.

Memory aid: *Storm = conflict weather.*

12. A trainee observes group stage in which cohesion, trust, norms and cooperation consolidate. This is an example of:

A. Termination and follow-up

B. Norming stage

C. Treatment plan

D. Progress review

Answer: B - Norming stage. Norming is marked by clearer expectations and stronger cohesion.

Memory aid: *Norming = norms settle.*

13. Which counseling/group/cultural concept is BEST described by: mature working stage in which members use the group productively toward goals?

A. Open-ended questioning

B. Performing stage

C. Adjourning stage

D. Cultural competence

Answer: B - Performing stage. Performing is the task-focused, therapeutically productive phase.

Memory aid: *Performing = productive work.*

14. Which counseling/group/cultural concept is BEST described by: ending stage involving review, separation, consolidation and transfer of learning?

A. Adjourning stage

B. Norming stage

C. Intervention stage

D. Progress review

Answer: A - Adjourning stage. Adjourning addresses termination and helps members carry gains beyond the group.

Memory aid: *Adjourning = goodbye and generalize gains.*

15. Which counseling/group/cultural concept is BEST described by: creating safety, trust, empathy and collaborative engagement at the beginning of counseling?

A. Active listening

B. Therapeutic confrontation

C. Assessment

D. Rapport building

Answer: D - Rapport building. Rapport supports disclosure and collaboration; it begins early but continues throughout treatment.

Memory aid: *RAPPORT starts with Respect, Attention, Presence.*

SECTION B: MODEL SHORT ESSAYS

1. Differentiate paraphrasing from reflection of feeling. (4mks)

- Paraphrasing restates the content/meaning in new words.

- Reflection of feeling identifies and feeds back the emotion or emotional meaning.

Memory aid: *Paraphrase = facts/meaning; reflection = feeling.*

2. Why should a qualified interpreter be preferred to a child family member? (4mks)

- Improves accuracy and reduces role burden on the child.

- Better supports confidentiality, informed consent and sensitive disclosure.

Memory aid: *Accuracy + confidentiality + appropriate role.*

3. State four tasks of ethical termination. (2mks)

- Review progress and goals.
- Discuss feelings and reasons for ending.
- Consolidate skills/relapse prevention.
- Arrange follow-up/referral/continuity when needed.

Memory aid: *RECAP*.

SUPERVISED VOCATIONAL QUALIFICATION DIPLOMA - EXAMINATION

Session: Revision Mock Set 03	Duration: 1hr 30mins
Specialty: Psychotherapy	Coefficient: 04
Eliminatory mark: < 14/20	Training reference: VTI-AEDA examination style

SIMULATION OF REAL WORKING SITUATION PAPER - SOLUTION / MARKING GUIDE PRACTICE SET 03

Revision material based on the supplied VTI-AEDA 2025 examination format and the supplied psychotherapy course recordings. Not an official examination paper.

DETAILED MODEL ANSWERS - SIMULATION OF REAL WORKING SITUATIONS

SECTION I

1. Trauma symptoms and stabilization - anchor: PTSD/trauma presentation - safety, grounding and stabilization before intensive processing when dissociation is high

After a violent incident, Nora, 39, experiences nightmares, intrusive memories, avoidance, hypervigilance and episodes of feeling detached or unreal. she becomes overwhelmed when asked to describe the event in detail.

Model response / marking points (25 marks):

1. Assess trauma symptoms, duration, functioning, suicide/self-harm risk, substance use and current safety.
2. Recognize intrusion, avoidance, negative mood/cognition and arousal; assess dissociation.
3. Explain trauma responses and normalize without minimizing.
4. Use grounding/orientation to present, paced breathing and stabilization skills.
5. Establish a window of tolerable arousal and stop/modify processing if dissociation increases.
6. Identify supports and coping resources.
7. Collaboratively set goals and consent for later trauma processing.
8. Use evidence-based trauma-focused treatment only when sufficiently stable and within competence.
9. Coordinate psychiatric/medical care where needed.
10. End with reorientation and a clear safety/follow-up plan.

Suggested mark allocation: Award marks for correct formulation/approach, risk and ethical priorities, logical session structure, appropriate technique application, collaboration/homework/follow-up, and professional communication. Credit clinically sound alternatives when clearly justified. Do not award full marks for naming a theory without showing how it is applied to the facts of the vignette.

Memory aid: SAFER: Safety, Assess, Frame/psychoeducate, Establish grounding, Review readiness.

2. Adolescent substance use - anchor: Substance-use assessment with motivational, supportive and safety-focused intervention

Patrick, age 16, has begun using pills and alcohol several times a week after being bullied and reports skipping school. The adolescent says, "It is the only thing that calms me down," and is unsure about stopping.

Model response / marking points (25 marks):

1. Explain confidentiality and its limits for a minor in the relevant setting.
2. Assess substances, quantity/frequency, intoxication/withdrawal, overdose risk, source, polysubstance use and impairment.
3. Assess suicide/self-harm, violence, exploitation and bullying safety.
4. Explore pros/cons and ambivalence without confrontation or shaming.
5. Link use to triggers and functions such as coping with bullying.
6. Elicit the client's own reasons for change and strengths.

7. Provide age-appropriate psychoeducation/harm reduction within scope.
8. Involve caregivers/other professionals according to consent, safety and legal requirements.
9. Refer for specialist substance/medical evaluation when indicated.
10. Create a concrete follow-up and safety plan.

Suggested mark allocation: Award marks for correct formulation/approach, risk and ethical priorities, logical session structure, appropriate technique application, collaboration/homework/follow-up, and professional communication. Credit clinically sound alternatives when clearly justified. Do not award full marks for naming a theory without showing how it is applied to the facts of the vignette.

Memory aid: ASK: Assess use/risk, Seek motivation, Keep safety/referral central.

SECTION II

1. Psychosis and supportive psychotherapy - anchor: Psychosis - supportive engagement, risk assessment and psychiatric collaboration

Lilian, 53, has a history of psychosis and recently began sleeping very little, withdrawing from family and saying that neighbors are sending coded messages through the radio. She is suspicious but willing to talk.

Model response / marking points (25 marks):

1. Use calm, respectful engagement and simple clear communication.
2. Assess hallucinations/delusions, disorganization, sleep, medication adherence, substance use and functional decline.
3. Assess suicide, violence, command hallucinations, inability to care for self and acute medical risk.
4. Do not ridicule or aggressively argue with the belief; validate emotion without confirming the delusion.
5. Orient to shared reality gently and focus on distress/coping.
6. Encourage medication/psychiatric review and coordinate with the treatment team.
7. Mobilize family/supports with appropriate consent and safety considerations.
8. Use simple coping and routine goals.
9. Escalate to emergency care if risk or severe decompensation requires it.
10. Document assessment and plan.

Suggested mark allocation: Award marks for correct formulation/approach, risk and ethical priorities, logical session structure, appropriate technique application, collaboration/homework/follow-up, and professional communication. Credit clinically sound alternatives when clearly justified. Do not award full marks for naming a theory without showing how it is applied to the facts of the vignette.

Memory aid: CALM: Connect, Assess risk, Listen without endorsing delusion, Mobilize care.

2. Eating disorder assessment - anchor: Anorexia nervosa presentation - multidisciplinary assessment plus psychotherapy

Benoit, 60, has lost substantial weight, restricts food, fears weight gain and exercises excessively. Family members report dizziness and fainting. Benoit insists there is no problem.

Model response / marking points (25 marks):

1. Recognize low-weight restriction/fear of weight gain as consistent with anorexia nervosa.
2. Assess eating patterns, compensatory behavior, body image, exercise, weight trajectory and motivation.
3. Assess medical instability urgently (fainting, dehydration, cardiac risk) and refer for medical evaluation.
4. Assess suicide/self-harm and comorbid depression/anxiety.
5. Use nonjudgmental engagement; avoid power struggles over food.
6. Use psychoeducation and motivational work.
7. Coordinate nutritional/medical care and family involvement where appropriate.
8. Use evidence-based psychotherapy within competence (e.g., CBT-oriented methods/family-based approaches depending on age/context).
9. Set measurable behavioral and psychological goals.

10. Monitor progress and medical risk closely.

Suggested mark allocation: Award marks for correct formulation/approach, risk and ethical priorities, logical session structure, appropriate technique application, collaboration/homework/follow-up, and professional communication. Credit clinically sound alternatives when clearly justified. Do not award full marks for naming a theory without showing how it is applied to the facts of the vignette.

Memory aid: *MIND: Medical risk, Intake/eating pattern, Negative body beliefs, Danger/suicide.*